

AMT2

Service Line Development

Armored Drill

8 concepts · 12 items · 4 transfer items

What this is. Every concept in this course's public description, re-asked with more disguise than a practice test uses. Each stem carries facts that do not decide it. Every wrong answer is a named trap.

Source. Scope from the course's public description in the WGU Institutional Catalog (September 2026); items written from the field's standard body of knowledge. No WGU assessment item, course material or proprietary content is used or reproduced. Not affiliated with or endorsed by Western Governors University.

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How to run it

Paper first. Pencil. One pass, no notes, a timer if you want the pressure.

1. **Before you read the options**, fill the strip under the item: the target word (what is actually being asked), and the one fact in the stem that decides it.
2. **Principle before arithmetic**. Name the rule, then compute.
3. **Camouflage**. Most of every stem does not matter. Ask: which single fact decides this?
4. **Inverse pairs are the bait**. If two options are opposites, one of them is the trap built for you.
5. **Mark sure, unsure or guess** honestly. Sure-and-wrong is the most valuable miss on the page — it is a wrong mental model.
6. **Absolutes** (always, never, guarantee) are usually wrong. And recognizing an answer is not the same as owning it.
7. Items with a **heavy black bar** on the left are **transfer items**: the same principle, reversed or solved for a different variable.
8. Stop at the STOP page. Then grade, fill the triage grid, and tally the traps you fell for. The biggest tally is what you drill next.

The strip, filled in, looks like this:

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	_____	

Items

1.

A community hospital is considering a new orthopedic service line. A well-known joint-replacement surgeon recently joined the staff and is eager to launch it, and the hospital just finished a new parking deck. What is the best first evidence that the service line is needed?

- A. The fact that two nearby competitors already offer orthopedics.
- B. The new surgeon's enthusiasm and past volumes elsewhere.
- C. None needed; demand always follows new capacity.
- D. Service-area data on population by age, projected procedure rates, current volumes and competitors' capacity.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

2.

A hospital's six-bed sleep lab, led by a board-certified sleep physician, earned \$900,000 in revenue last year with \$600,000 in variable costs. It was also allocated \$400,000 of hospital overhead, which would remain if the lab closed. The report shows a \$100,000 loss. Should the hospital close the lab to improve overall profit?

- A. Yes; closing removes the \$100,000 loss.
- B. No; the lab brings in \$900,000 in revenue.
- C. Yes; closing saves \$1,000,000 in costs.
- D. No; closing would lower hospital profit by \$300,000.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

3. [TRANSFER — reversed stem / different variable]

A service line's contribution margin is \$250,000. If it closes, \$300,000 of its fixed costs would also go away. How would closing it change hospital profit?

- A. Profit rises by \$300,000.
- B. Profit falls by \$50,000.
- C. Profit rises by \$50,000.
- D. Profit falls by \$250,000.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

4.

A hospital plans a 12-chair outpatient infusion center designed by an award-winning architect. Fixed costs would be \$1,200,000 a year. Average reimbursement is \$500 per visit and variable cost is \$200 per visit. How many visits a year does the center need to break even?

- A. About 1,714
- B. 2,400
- C. 6,000
- D. 4,000

Target word _____	The one fact that decides _____	Answer _____	[] sure [] unsure [] guess
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5. [TRANSFER — reversed stem / different variable]

A clinic breaks even at 5,000 visits a year. Fixed costs are \$750,000 and variable cost is \$90 per visit. What average reimbursement per visit does this imply?

- A. \$240
- B. \$150
- C. \$60
- D. \$90

Target word _____	The one fact that decides _____	Answer _____	[] sure [] unsure [] guess
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6.

A hospital imaging department performed a record 24,000 exams last year using 12,000 worked hours. Its new director came from an academic medical center. The productivity target is 0.45 worked hours per exam. How did the department perform against the target?

- A. It met the target, because volume was a record.
- B. 2.0 exams per hour; it beat the 0.45 target.
- C. 0.50 hours per exam; it missed the target by using more labor per exam.
- D. 0.50 hours per exam; it beat the target because the number is higher.

Target word _____	The one fact that decides _____	Answer _____	[] sure [] unsure [] guess
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7.

In a hospital's service area there were 8,000 births last year. The hospital, whose NICU recently earned a higher level designation, delivered 2,000. Its main competitor delivered 3,500, and three smaller hospitals delivered the rest. What is the hospital's market share for births?

- A. About 57%
- B. 25%
- C. About 36%
- D. About 44%

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

8. [TRANSFER — reversed stem / different variable]

A hospital wants a 30% share of a market with 12,000 procedures a year. It currently performs 2,700. How many more procedures a year does it need?

- A. 900
- B. 9,300
- C. 3,600
- D. 810

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

9.

A hospital is negotiating with an independent cardiology group to staff a new cardiac service line. The group, which has practiced in the region for 30 years, opened by asking for \$4 million a year. If talks fail, the hospital can recruit and employ two cardiologists for a total cost of \$2.6 million a year with similar quality. What should set the hospital's walk-away point?

- A. Its BATNA: the cost and quality of recruiting its own cardiologists.
- B. Halfway between the two figures.
- C. There is none; the hospital should never walk away once talks begin.
- D. The group's \$4 million opening request.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

10.

A hospital's 24-bed orthopedic unit, staffed by hospital-employed surgeons, is paid a fixed \$15,000 per joint replacement under a bundled payment. Shortening the average stay from three days to two cuts the cost per case from \$13,000 to \$11,500. What happens to the margin per case?

- A.** It falls, because there are fewer billable days.
- B.** It rises from \$2,000 to \$3,500.
- C.** It becomes a \$1,500 loss per case.
- D.** It stays the same, because the payment is fixed.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

11. [TRANSFER — reversed stem / different variable]

A payer pays a hospital a fixed amount per day of inpatient stay. If nothing else changes, what happens to revenue per case when length of stay is shortened?

- A.** It rises.
- B.** It stays the same, because payment is fixed per case.
- C.** It falls, because fewer days are paid.
- D.** Margin always improves when stays are shorter.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

12.

A health system in a state with a certificate-of-need law plans to build a \$12 million freestanding imaging center. The system's board chair is a local banker, and the land is already owned. What must the system do before building?

- A.** Get only a local zoning permit.
- B.** Obtain certificate-of-need approval from the state by showing community need.
- C.** Nothing; private systems never need state approval to expand.
- D.** Earn accreditation for the center first.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

STOP

Answers and dissection follow. Finish every item and every strip first.

Key and dissection

Struck-through text is the camouflage: facts that do not decide the item.

1. Answer D Concept 1

A community hospital is considering a new orthopedic service line. ~~A well-known joint replacement surgeon recently joined the staff and is eager to launch it, and the hospital just finished a new parking deck.~~ What is the best first evidence that the service line is needed?

Target word: best first evidence of need **Decides:** Need is shown by market demand data.

A	Confusion set	Competitors' presence may mean the market is already served.
B	Camouflage	Answers the surgeon detail; one physician's interest is not market need.
C	Absolute language	Unused capacity is a common cause of service-line losses.
D	Correct	A needs assessment sizes unmet demand in the market.

2. Answer D Concept 2

A hospital's ~~six-bed sleep lab, led by a board-certified sleep physician,~~ earned \$900,000 in revenue last year with \$600,000 in variable costs. It was also allocated \$400,000 of hospital overhead, which would remain if the lab closed. The report shows a \$100,000 loss. Should the hospital close the lab to improve overall profit?

Target word: close or keep **Decides:** The lab contributes \$300,000 toward overhead that would stay anyway.

A	Dropped element	Drops the fact that the allocated overhead stays after closure.
B	Wrong stem number	Revenue alone does not decide it; contribution margin does.
C	Adjacent category	Counts overhead that will not go away and ignores lost revenue.
D	Correct	Contribution margin $\$900,000 - \$600,000 = \$300,000$ would be lost; the \$400,000 overhead remains.

3. Answer C Concept 2 • transfer

A service line's contribution margin is \$250,000. If it closes, \$300,000 of its fixed costs would also go away. How would closing it change hospital profit?

Target word: profit change from closing **Decides:** Avoidable fixed costs exceed the contribution margin.

A	Wrong stem number	Counts the savings but not the lost margin.
B	Inverted operation	Right size, wrong direction.
C	Correct	Save \$300,000, lose \$250,000: net +\$50,000.
D	Dropped element	Drops the avoidable fixed costs.

4. Answer D Concept 3

A hospital plans a ~~12-chair~~ outpatient infusion center ~~designed by an award-winning architect.~~ Fixed costs would be \$1,200,000 a year. Average reimbursement is \$500 per visit and variable cost is \$200 per visit. How many visits a year does the center need to break even?

Target word: break-even visits **Decides:** Contribution margin = $\$500 - \$200 = \$300$ per visit.

A	Adjacent category	$\$1,200,000 / \700 adds variable cost to payment.
B	Dropped element	$\$1,200,000 / \500 drops the variable cost.
C	Inverted operation	$\$1,200,000 / \200 divides by variable cost instead of margin.
D	Correct	$\$1,200,000 / \$300 = 4,000$ visits.

5. Answer A Concept 3 • transfer

A clinic breaks even at 5,000 visits a year. Fixed costs are \$750,000 and variable cost is \$90 per visit. What average reimbursement per visit does this imply?

Target word: solve for reimbursement **Decides:** Margin per visit = $\$750,000 / 5,000 = \150 .

A	Correct	$\$150 \text{ margin} + \$90 \text{ variable cost} = \240 .
B	Dropped element	That is the margin; the variable cost must be added.
C	Inverted operation	Subtracts variable cost from the margin.
D	Wrong stem number	That is the variable cost.

6. Answer C Concept 4

A hospital imaging department performed ~~a record~~ 24,000 exams last year using 12,000 worked hours. ~~Its new director came from an academic medical center.~~ The productivity target is 0.45 worked hours per exam. How did the department perform against the target?

Target word: productivity vs. target **Decides:** $12,000 / 24,000 = 0.50$ hours per exam, above the 0.45 target.

A	Camouflage	Record volume does not decide labor per exam.
B	Confusion set	Compares exams per hour with a target stated in hours per exam.
C	Correct	Lower hours per unit is better; 0.50 is worse than 0.45.
D	Inverted operation	For hours per unit, higher means less productive.

7. Answer B Concept 5

In a hospital's service area there were 8,000 births last year. The hospital, ~~whose NICU recently earned a higher level designation,~~ delivered 2,000. Its main competitor delivered 3,500, and three smaller hospitals delivered the rest. What is the hospital's market share for births?

Target word: market share **Decides:** Hospital volume / total market volume.

A	Confusion set	$2,000 / 3,500$ divides by the competitor's volume.
B	Correct	$2,000 / 8,000 = 25\%$.
C	Dropped element	$2,000 / 5,500$ drops the three smaller hospitals.
D	Wrong stem number	$3,500 / 8,000$ is the competitor's share.

8. Answer A Concept 5 • transfer

A hospital wants a 30% share of a market with 12,000 procedures a year. It currently performs 2,700. How many more procedures a year does it need?

Target word: additional volume needed **Decides:** Target volume = $30\% \times 12,000 = 3,600$.

A	Correct	$3,600 - 2,700 = 900$.
B	Adjacent category	All the market volume it does not have.
C	Dropped element	The target volume, without subtracting current volume.
D	Confusion set	30% of current volume, not of the market.

9. Answer A Concept 6

A hospital is negotiating with an independent cardiology group to staff a new cardiac service line. The group, ~~which has practiced in the region for 30 years,~~ opened by asking for \$4 million a year. If talks fail, the hospital can recruit and employ two cardiologists for a total cost of \$2.6 million a year with similar quality. What should set the hospital's walk-away point?

Target word: walk-away point **Decides:** The hospital's best alternative costs \$2.6 million.

A	Correct	A deal worse than the best alternative should be declined.
B	Adjacent category	Splitting the difference is a tactic, not a walk-away point.
C	Absolute language	Without a walk-away point, any deal must be accepted.
D	Confusion set	An opening anchor is not the hospital's limit.

10. Answer B Concept 7

A hospital's ~~24-bed~~ orthopedic unit, ~~staffed by hospital-employed surgeons,~~ is paid a fixed \$15,000 per joint replacement under a bundled payment. Shortening the average stay from three days to two cuts the cost per case from \$13,000 to \$11,500. What happens to the margin per case?

Target word: margin change **Decides:** Payment is fixed per case; cost falls.

A	Confusion set	Applies per-diem logic; the bundle pays per case.
B	Correct	$\$15,000 - \$13,000 = \$2,000$; $\$15,000 - \$11,500 = \$3,500$.
C	Inverted operation	Treats the cost savings as a loss.
D	Dropped element	Drops the lower cost.

11. Answer C Concept 7 • transfer

A payer pays a hospital a fixed amount per day of inpatient stay. If nothing else changes, what happens to revenue per case when length of stay is shortened?

Target word: revenue under per-diem **Decides:** Payment is per day.

A	Inverted operation	Fewer paid days cannot raise per-diem revenue.
B	Confusion set	That describes DRG or bundled payment.
C	Correct	Per-diem revenue rises and falls with days.
D	Absolute language	Under per-diem payment, revenue drops too, so margin can fall.

12. Answer B Concept 8

A health system in a state with a certificate-of-need law plans to build a \$12 million freestanding imaging center. ~~The system's board chair is a local banker, and the land is already owned.~~ What must the system do before building?

Target word: required step **Decides:** The state has a CON law covering major capital projects.

A	Adjacent category	Zoning is needed but does not replace CON approval.
B	Correct	CON states require approval before covered new facilities or major capital spending.
C	Absolute language	CON laws apply to private providers in those states.
D	Confusion set	Accreditation reviews quality of operation, not approval to build.

Triage grid

Write each item number in the box it belongs to.

	Right	Wrong
Sure	Owned.	Concept inversion — top priority. A wrong mental model; it survives rewording.
Unsure / guess	Recognition, not mastery. Re-drill with the transfer.	Not learned yet.

Trap tally

For every miss, find the letter you chose in the key and tally its trap. The biggest number is the pattern to drill.

Trap	What it does	In this drill	Your misses
Dropped element	Leaves out a required term or step.	7	
Wrong stem number	A figure copied from the prompt that is not the answer.	4	
Adjacent category	The right idea in the wrong bucket.	5	
Inverted operation	Direction flipped: add/subtract, cause/effect, higher/lower.	6	
Absolute language	Always, never, guarantee — where judgment applies.	4	
Confusion set	Every option is real; the test is exact role-matching.	8	
Camouflage	Answers a stem fact that was never the question.	2	

Anchor rows — the concept list

One row per concept: the rule to lock in, the trap that catches people, and the tell in the question that signals it.

#	Concept	Rule	Trap	Tell
1	Needs assessment	Justify a new service line with market evidence: service-area population, use rates, current volumes, competitor capacity and unmet demand.	Launching because a physician is eager or a competitor has one.	A proposed service and a question about the best evidence to start with.
2	Contribution margin by service line	Keep a line if its contribution margin (revenue – variable cost) exceeds the fixed costs that would disappear if it closed; allocated overhead that stays does not count.	Closing a line that shows a loss only because of allocated overhead.	A service-line loss that includes fixed costs that would remain after closure.
3	Break-even volume	Break-even volume = fixed costs / (payment per unit – variable cost per unit).	Dividing by payment alone or by variable cost.	Annual fixed costs, average reimbursement and variable cost per case.
4	Labor productivity	Worked hours per unit of service = worked hours / units; lower is more productive.	Reading a higher hours-per-unit figure as better, or mixing units.	Worked hours, volume and a productivity target.
5	Market share	Market share = the organization's volume / total market volume in the service area.	Dividing by one competitor's volume or leaving out part of the market.	Total service-area volume and several providers' volumes.
6	BATNA in negotiation	Your walk-away point is set by your best alternative to a negotiated agreement.	Letting the other side's opening number set your limit.	A negotiation and a known fallback option.
7	Payment model incentives	Under fixed per-case payment (DRG or bundle), lowering cost per case raises margin; under per-diem or fee-for-service, more days or services raise revenue.	Applying fee-for-service logic to a fixed case payment, or the reverse.	How the payer pays: per case, per day, or per service.
8	Certificate of need	In states with certificate-of-need laws, providers must get state approval, showing community need, before certain new facilities, services or major capital spending.	Assuming a zoning permit or accreditation is enough to build.	A new facility or major capital project in a CON state.

Course scope (public description): Service Line Development will address how to critically assess the competitive marketplace as well as the internal environment to establish a new line of business. Topics include needs assessment, international healthcare trends, service line management, revenue analysis, costs and productivity, communication, negotiation, health policy, health legislation, and facilities management, which are variables in the evaluation process.